

1. Administrative & Study Identification

Full Study Title: Real World Efficiency of Abrocitinib Treatment at Patients With Moderate to Severe Atopic Dermatitis Who Had Inadequate Response to Previous Biologic Therapies
Official Title: A Prospective, Multi-center Observational Study Characterizing Clinical Outcomes of Patients Receiving Abrocitinib for Moderate-to-severe Atopic Dermatitis Who Had an Inadequate Response (or Intolerance) to ≤2 Previous Biologic Therapies Approved for Moderate-to-severe Atopic Dermatitis
Protocol Number: Pfizer Internal Protocol / Observational
NCT Number: NCT06899204
Trial Status: Not Yet Recruiting
Sponsor/Company Name: Pfizer
Investigational Product: Abrocitinib (JAK1 inhibitor)
Phase of Development: Not Applicable (Observational)
Medical Monitor: Pfizer Clinical Operations Medical Monitor

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To measure the effectiveness of abrocitinib in a real-world setting in adult and adolescent patients (aged ≥12 years) with moderate-to-severe atopic dermatitis (M2S AD), with an inadequate response or intolerance to ≤ 2 previous biologic therapies. **Secondary Objectives:** To characterize clinical and patient-reported outcomes (PROs) in this specific patient population.

2.2 Background & Rationale To address the gap in real-world effectiveness data for abrocitinib in patients with moderate-to-severe atopic dermatitis who have previously experienced an inadequate response or intolerance to up to 2 biologic therapies approved for M2S AD in the United States.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Prospective, multi-center)
Control Type: Single-arm (All participants receive the prescribed intervention)
Blinding: Open-label
Randomization: N/A (No sampling; consecutive recruitment of eligible patients)

3.2 Planned Enrollment & Duration Target N: 150 patients
Number of Sites: Approximately 15 sites across the US

Enrollment Period: Open for approximately 12 months after the first patient has been enrolled (subject to reassessment based on real-world enrollment rates).

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria Patients must meet all of the following inclusion criteria: 1. Participants who have chronic AD that has been present for ≥ 1 year before screening. 2. Male and female adult and adolescent patients aged ≥ 12 years (>12 years at baseline). 3. Patients with a diagnosis of moderate-to-severe atopic dermatitis confirmed by a certified dermatologist, who are prescribed abrocitinib for use in accordance with the product label (USPI) and independently of the decision to enroll the patient in this study. 4. Patients who have inadequate responses or are intolerant to ≤ 2 previous biologic therapies approved for M2S AD. (Patients shall have had an inadequate response and/or intolerance to at least one, but no more than 2 biologic therapies approved for moderate-to-severe AD). 5. Evidence of a personally signed and dated informed consent document indicating that the patient (or a legally acceptable representative) has been informed of all pertinent aspects of the study. Adolescents must provide assent, and one or both parents/guardians must provide signed informed consent before any study-related activity. 6. Patients who, in the opinion of the investigator, are willing and able to comply with regular clinic visits as per standard practice at the site and agree to complete PRO questionnaires and other patient-completed questions.

4.2 Exclusion Criteria Patients meeting any of the following criteria will not be included: 1. Patients that currently have active forms of other inflammatory skin diseases, other than AD, or have evidence of skin conditions (e.g., psoriasis, seborrheic dermatitis, Lupus) at the time of Day 1 that would interfere with evaluation of atopic dermatitis or response to treatment. 2. Patients previously treated with abrocitinib or other oral/systemic JAK inhibitors. 3. Investigator site staff or Pfizer employees directly involved in the conduct of the study, site staff otherwise supervised by the investigator, and their respective family members. 4. Patient eligibility should be reviewed, documented, and confirmed by an appropriately qualified member of the investigator's study team before patients are enrolled in the study.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Abrocitinib oral tablets (prescribed independently by the physician in accordance with the USPI product label).

Route of Administration: Oral

Duration of Treatment: Observational period across regular clinical intervals.

5.2 Concomitant & Prohibited Therapy Permitted: Standard topical AD therapies or concomitant care as dictated by routine clinical practice.

Prohibited: Other investigational therapies or previous use of systemic JAK inhibitors prior to study enrollment.

6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :---
| :--- | :--- | | **Screening / Baseline** | Day 0 | Informed Consent, Medical History, AD Diagnosis Confirmation, e-diary setup, Prescribing Abrocitinib | | **Interim Visits** | Weeks 2, 4, 16 | Regular clinic visits: Assessment of EASI, vIGA, BSA, and collection of PRO/e-diary data | | **End of Study** | Study Completion | Final clinic visit, completion of all patient-reported outcome assessments |

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes 1. **EASI-75 Response:** Percentage of patients achieving EASI-75 improvement from baseline at Week 16 after the index date. (*Note: EASI evaluates severity based on clinical signs [erythema, induration/papulation, excoriation, lichenification] and % of body surface area [BSA] affected across 4 body regions, yielding a total score from 0.0 to 72.0*). 2.

Pruritus Improvement: Percentage of patients achieving ≥ 4 -point improvement in Peak Pruritus Numerical Rating Scale (PP-NRS) collected daily all through the study from baseline at Week 2 by e-diary assessment. (*Note: NRS ranges from 0 [no itch] to 10 [worst itch imaginable]*).

7.2 Secondary Outcomes 1. **EASI-75, 90, and 100 Responses:** Percentage of patients achieving EASI-75, 90, and 100 from baseline at Week 4 and 16 after the index date. 2. **v-IGA Response:** Percentage of patients achieving Validated Investigator Global Assessment (v-IGA) response of Clear (0) or Almost Clear (1) and ≥ 2 points improvement from baseline at Week 4 and 16 after the index date. 3. **BSA Change:** Change from baseline in total percentage of Body Surface Area (BSA) affected at Week 4 and 16.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via regular clinic visits and patient-reported questionnaires.

Serious Adverse Events (SAEs): Routine reporting timeline (typically 24 hours to the sponsor).

Data Monitoring Committee (DMC): Internal safety monitoring mechanisms managed by Pfizer.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Includes all enrolled patients receiving at least one dose of abrocitinib who satisfy eligibility criteria.

Sample Size Justification: \$N=150\$ patients determined sufficient to adequately characterize real-world clinical and PRO data across the target population, driven by real-world enrollment rates.

Handling of Missing Data: Standard handling for observational data; outcomes evaluated strictly based on real-world visit adherence.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP), the Declaration of Helsinki, and real-world evidence data guidelines.

Monitoring Plan: Centralized/remote monitoring of electronic records, e-diaries, and PRO questionnaire completion.

Audit Trail: Documented tracking of eCRFs and patient questionnaires via validated systems.

11. Study Identifiers & Governance

Primary ID: Pfizer Internal Identifier

Registry Number: NCT06899204

Posting ID: 1050492054222593935

Sponsor/Lead Organization: Pfizer

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12. Clinical Framework (Atopic Dermatitis Specific)

Disease Areas: Atopic Dermatitis, Atopic Dermatitis Unspecified, Dermatitis (Atopic)

Diagnosis Threshold: Chronic AD ≥ 1 year; inadequate response/intolerance to 1 or 2 previous biologics approved for M2S AD

Medical Methodology: Standard clinical observation while receiving oral Abrocitinib

Optimization Target: Achievement of clear/almost clear skin (v-IGA 0 or 1), EASI improvements (75/90/100), and meaningful improvement in daily pruritus (PP-NRS).

13. Study Procedures & Methodology

Management Pathway: Real-world standard clinical practice management for AD

Education Protocol (HCP): N/A (observational standard care)

Education Protocol (Patient): Self-reporting directions for e-diaries and PROs (e.g., daily PP-NRS assessments)

Quality Audit Mechanism: Regular clinic visits ensuring completion of standardized clinical surveys and physician assessments

14. Observation & Follow-Up Schedule

Total Duration: 12-month enrollment window; individual follow-up includes key assessments at Week 2, Week 4, and Week 16

Onsite Visit Cadence: Regular clinic visits as dictated by standard medical practice

Remote Monitoring Frequency: Continuous daily e-diary capture for Pruritus NRS; intermittent review of PRO data submissions

Checklist Review Interval: Questionnaires and scale assessments (EASI, v-IGA, BSA) captured aligned with respective clinic visits

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				